

D346

Advanced Psychiatric Mental Health Care of Adults and Older Adults Across Care Settings

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 79-year-old retired postal worker in a nursing home becomes confused over two days. Her attention drifts, and she is alert in the morning but drowsy and disoriented at night. She has burning urination. Her family says she loved crossword puzzles. What is the priority?

- A. Start an antipsychotic at night to control confusion.
- B. Diagnose Alzheimer's disease and start a cholinesterase inhibitor.
- C. Accept it as normal aging.
- D. Treat it as delirium and look for and treat the underlying cause, such as a urinary tract infection.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Over three years, a 74-year-old has gradually lost memory for recent events and now struggles to manage bills. He is fully alert and his attention is steady during the exam. Which is most likely?

- A. Depression, since all memory problems in older adults are depression
- B. Major neurocognitive disorder (dementia)
- C. Delirium
- D. Normal aging

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A 36-year-old real estate agent with bipolar I disorder presents with a major depressive episode. He had one manic hospitalization five years ago and drinks socially on weekends. What is the most appropriate treatment approach?

- A. Treat only the weekend drinking first.
- B. No medication, because his mania was five years ago.
- C. A mood stabilizer or an antipsychotic with evidence for bipolar depression, avoiding antidepressant monotherapy.
- D. An SSRI alone, because this episode is depressive.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

Three weeks after starting an SSRI alone for depression, a patient sleeps 3 hours a night without feeling tired, talks rapidly, and spends heavily. What most likely happened?

- A.** Normal SSRI activation that always fades.
- B.** A good response to the SSRI.
- C.** Serotonin syndrome.
- D.** An antidepressant-associated switch into mania or hypomania, suggesting a bipolar spectrum disorder.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 30-year-old man with schizophrenia, who lives in a group home and enjoys watching baseball, shows little facial expression, speaks very little, and has stopped bathing or starting any activities. His hallucinations are controlled. Which symptoms remain?

- A.** Negative symptoms: flat affect, alogia, and avolition.
- B.** Side effects of watching too much TV.
- C.** Catatonia
- D.** Positive symptoms.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

Which finding is a positive symptom of schizophrenia?

- A.** Hearing voices commenting on one's actions.
- B.** Loss of motivation.
- C.** Poor working memory.
- D.** Reduced speech output.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A 45-year-old firefighter with 20 years of service has PTSD after a fatal house fire, with nightmares, avoidance, and hypervigilance. He asks for a sleeping pill like his neighbor uses. What is the first-line treatment?

- A. A trauma-focused psychotherapy such as cognitive processing therapy or prolonged exposure.
- B. Whatever sleep aid his neighbor uses.
- C. Wait, since PTSD always resolves on its own.
- D. A benzodiazepine for sleep and anxiety.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A 78-year-old widower recently retired from farming and lives alone with a hunting rifle at home. He says, “It doesn’t matter much anymore.” He grows tomatoes every summer. What should the PMHNP do?

- A. Assess only if he states a specific plan.
- B. Treat the comment as normal for his age.
- C. Encourage him to keep gardening and follow up next year.
- D. Ask directly about suicidal thoughts and plans and counsel on lethal means safety for the rifle.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

An 84-year-old with Alzheimer’s disease in a memory care unit becomes agitated every afternoon and paces. She was a piano teacher for 40 years. Staff request an antipsychotic. What is the best first step?

- A. Never use any medication in dementia.
- B. Look for triggers (pain, hunger, noise) and use non-drug strategies such as routine and structured activity.
- C. Start an antipsychotic at a regular adult dose.
- D. Add a benzodiazepine as needed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A 25-year-old with schizophrenia has relapsed three times in a year. He lives with his parents, who often criticize him and closely monitor everything he does. The family recently moved to a larger house. Which intervention most directly addresses his relapse risk?

- A.** Medication changes alone.
- B.** Family psychoeducation to reduce criticism and over-involvement.
- C.** Attributing relapses to the move.
- D.** Asking parents to monitor him even more closely.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A 52-year-old admitted for alcohol withdrawal has a CIWA-Ar score of 18. He is poorly nourished, and an IV with dextrose has been ordered. He works as a sales manager. Which order of care is correct?

- A.** Use an antipsychotic alone to control withdrawal.
- B.** Start dextrose first, then give thiamine tomorrow.
- C.** Give thiamine before dextrose and treat withdrawal with a benzodiazepine per protocol.
- D.** Withhold treatment until symptoms become severe.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

About 72 hours after his last drink, a patient becomes disoriented with fever, rapid heart rate, heavy sweating, and visual hallucinations. What is most likely?

- A.** Withdrawal seizure
- B.** Wernicke encephalopathy only
- C.** Delirium tremens
- D.** Alcoholic hallucinosis

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 79-year-old ~~retired postal worker~~ in a nursing home becomes confused over two days. Her attention drifts, and she is alert in the morning but drowsy and disoriented at night. She has burning urination. ~~Her family says she loved crossword puzzles.~~ What is the priority?

Target word: delirium recognition **Decides:** Acute onset, fluctuating attention, likely medical cause.

A	Dropped element	Skips finding the cause.
B	Confusion set	Dementia is gradual; this is acute.
C	Absolute language	Sudden confusion is never just normal aging.
D	Correct	Delirium is acute and fluctuating and needs a medical workup.

2. Answer B Concept 1 • transfer

Over three years, a 74-year-old has gradually lost memory for recent events and now struggles to manage bills. He is fully alert and his attention is steady during the exam. Which is most likely?

Target word: reverse: identify dementia **Decides:** Gradual onset, stable alertness and attention.

A	Absolute language	Depression can mimic dementia but not always.
B	Correct	Gradual, progressive decline with intact attention.
C	Inverted operation	Delirium is acute with fluctuating attention.
D	Adjacent category	Loss of ability to manage finances is functional decline.

3. Answer C Concept 2

A 36-year-old ~~real estate agent~~ with bipolar I disorder presents with a major depressive episode. He had one manic hospitalization five years ago ~~and drinks socially on weekends.~~ What is the most appropriate treatment approach?

Target word: bipolar depression treatment **Decides:** History of mania.

A	Camouflage	Social drinking does not change the core treatment decision.
B	Adjacent category	Bipolar disorder is lifelong and needs maintenance.
C	Correct	Reduces risk of switching to mania.
D	Dropped element	Ignores the bipolar history.

4. Answer D Concept 2 • transfer

Three weeks after starting an SSRI alone for depression, a patient sleeps 3 hours a night without feeling tired, talks rapidly, and spends heavily. What most likely happened?

Target word: reverse: from effect to cause **Decides:** Manic symptoms after antidepressant alone.

A	Absolute language	These symptoms meet manic features and need action.
B	Inverted operation	Decreased need for sleep and spending are not recovery.
C	Confusion set	No fever, clonus, or autonomic instability.
D	Correct	Needs reassessment and usually stopping the SSRI.

5. Answer A Concept 3

A 30-year-old man with schizophrenia, ~~who lives in a group home and enjoys watching baseball~~, shows little facial expression, speaks very little, and has stopped bathing or starting any activities. His hallucinations are controlled. Which symptoms remain?

Target word: symptom type **Decides:** Losses of normal function.

A	Correct	They are absences of normal function.
B	Camouflage	Hobbies do not explain these symptoms.
C	Adjacent category	Catatonia involves motor signs like stupor or posturing.
D	Inverted operation	Positive symptoms are added experiences like hallucinations.

6. Answer A Concept 3 · transfer

Which finding is a positive symptom of schizophrenia?

Target word: reverse: find the positive **Decides:** Added experience.

A	Correct	Hallucinations are positive symptoms.
B	Confusion set	Avolition is negative.
C	Adjacent category	A cognitive symptom.
D	Inverted operation	Alogia is negative.

7. Answer A Concept 4

A 45-year-old firefighter ~~with 20 years of service~~ has PTSD after a fatal house fire, with nightmares, avoidance, and hypervigilance. He asks for a sleeping pill ~~like his neighbor uses~~. What is the first-line treatment?

Target word: first-line PTSD treatment **Decides:** Trauma-focused psychotherapy.

A	Correct	Strongest evidence for PTSD.
B	Camouflage	Another person's medication does not guide care.
C	Absolute language	Many cases persist without treatment.
D	Adjacent category	Not recommended for PTSD; may worsen outcomes.

8. Answer D Concept 5

A 78-year-old widower recently retired from farming and lives alone with a hunting rifle at home. He says, "It doesn't matter much anymore." ~~He grows tomatoes every summer.~~ What should the PMHNP do?

Target word: late-life suicide risk **Decides:** Older man, losses, isolation, firearm, hopeless statement.

A	Dropped element	Warning signs call for assessment without waiting.
B	Inverted operation	Older men have the highest suicide rates.
C	Camouflage	Hobbies do not replace risk assessment.
D	Correct	High-risk profile needs direct assessment and means restriction.

9. Answer B Concept 6

An 84-year-old with Alzheimer's disease in a memory care unit becomes agitated every afternoon and paces. ~~She was a piano teacher for 40 years.~~ Staff request an antipsychotic. What is the best first step?

Target word: first-line for dementia agitation **Decides:** Non-drug approaches first; antipsychotics carry mortality risk.

A	Absolute language	Medication may be needed for severe, dangerous symptoms after other steps.
B	Correct	Non-pharmacologic care is first-line.
C	Dropped element	Skips non-drug steps and ignores the boxed warning.
D	Adjacent category	Raises falls and confusion in older adults.

10. Answer B Concept 7

A 25-year-old with schizophrenia has relapsed three times in a year. He lives with his parents, who often criticize him and closely monitor everything he does. ~~The family recently moved to a larger house.~~ Which intervention most directly addresses his relapse risk?

Target word: family factor in relapse **Decides:** High expressed emotion.

A	Dropped element	Leaves out the family factor.
B	Correct	Lowering expressed emotion reduces relapse.
C	Camouflage	Relapses preceded and are unrelated to the move.
D	Inverted operation	Increases over-involvement.

11. Answer C Concept 8

A 52-year-old admitted for alcohol withdrawal has a CIWA-Ar score of 18. He is poorly nourished, and an IV with dextrose has been ordered. ~~He works as a sales manager.~~ Which order of care is correct?

Target word: sequence of care **Decides:** Thiamine before glucose; benzodiazepine for significant withdrawal.

A	Adjacent category	Antipsychotics lower seizure threshold and do not treat withdrawal.
B	Inverted operation	Reverses the safe order.
C	Correct	Glucose first can precipitate Wernicke encephalopathy.
D	Dropped element	A score of 18 needs medication.

12. Answer C Concept 8 · transfer

About 72 hours after his last drink, a patient becomes disoriented with fever, rapid heart rate, heavy sweating, and visual hallucinations. What is most likely?

Target word: timeline to syndrome **Decides:** 48–96 hours with confusion and autonomic instability.

A	Confusion set	Typically 12–48 hours and presents as a seizure.
B	Dropped element	Wernicke causes confusion, ataxia, and eye signs, not this autonomic picture.
C	Correct	Occurs about 48–96 hours with disorientation and autonomic instability.
D	Adjacent category	Usually 12–48 hours with a clear sensorium and stable vitals.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owled.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	7	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	8	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	4	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Delirium vs. dementia	Delirium has acute onset, fluctuating course, and impaired attention and usually has a medical cause to find and treat; dementia (major neurocognitive disorder) is gradual with intact alertness early.	Attributing sudden confusion in an older adult to dementia or age.	Onset over hours to days and fluctuating attention.
2	Treating bipolar depression	In bipolar disorder, antidepressant monotherapy risks switching to mania; use a mood stabilizer or an antipsychotic with evidence for bipolar depression.	Treating any depressive episode with an SSRI alone.	Depression plus a history of mania or hypomania.
3	Positive vs. negative symptoms of schizophrenia	Positive symptoms add abnormal experiences (hallucinations, delusions, disorganization); negative symptoms are losses (flat affect, alogia, avolition, anhedonia, asociality).	Calling flat affect or withdrawal depression-only, or calling hallucinations negative because they are bad.	A symptom that is added vs. something that is missing.
4	First-line PTSD treatment	Trauma-focused psychotherapies (e.g., cognitive processing therapy, prolonged exposure) are first-line; benzodiazepines are not recommended for PTSD.	Using benzodiazepines for PTSD anxiety or nightmares.	Trauma-related symptoms and a treatment choice.
5	Late-life suicide risk	Older men have the highest suicide rates; risk rises with loss, isolation, illness, and firearm access, so assess directly and counsel on lethal means safety.	Assuming older adults are low risk or that talk of death is just normal aging.	An older adult with recent losses and access to lethal means.
6	Behavioral symptoms of dementia	Use non-drug approaches first for agitation in dementia; antipsychotics carry a boxed warning for increased death in older adults with dementia-related psychosis and are reserved for severe, dangerous symptoms.	Starting an antipsychotic as first-line for wandering or agitation.	Agitation in a person with dementia.
7	Family dynamics and relapse	High expressed emotion (criticism, hostility, emotional over-involvement) in families predicts relapse in schizophrenia and mood disorders; family psychoeducation reduces it.	Assuming more family involvement is always protective.	A family that is critical or overinvolved and a patient who keeps relapsing.

#	Concept	Rule	Trap	Tell
8	Alcohol withdrawal management	Withdrawal is monitored with CIWA-Ar and treated with benzodiazepines; give thiamine before glucose; seizures peak around 12–48 hours and delirium tremens around 48–96 hours after the last drink.	Giving glucose first or misreading the timeline.	Hours since last drink and a new symptom.

Course scope (public description): Advanced Psychiatric Care of Adults and Older Adults Across Care Settings prepares students to provide evidence-based mental healthcare for adults, older adults, and families. This course guides students through the application of age and developmentally appropriate advanced practice health assessment knowledge and diagnostic reasoning skills for adults, older adults, and families experiencing complex mental health issues. This course helps students develop treatment plans using psychotherapeutic treatment modalities, psychopharmacology, and community resources to manage specific mental health disorders for adults, older adults, and families. This course also includes the influences of family dynamics and societal norms on mental health progression and recovery. The following courses are prerequisites: All MSN Core courses, Advanced Pathophysiology for the Advanced Practice Nurse, Advanced Pharmacology for the Advanced Practice Nurse, and Advanced Health Assessment for the Advanced Practice Nurse.